Patient information sheet – Jaw surgery

What are the types of Corrective Jaw surgeries?

1. Lefort I maxillary osteotomy – In this operation, cuts are made in the upper jaw (maxilla) and the bone is moved forwards, backwards, upwards or downwards depending upon the deformity and then fixed with titanium plates and screws in the new position.

2. Bilateral sagittal split osteotomy of mandible (BSSO) – In this operation, cuts are made in the lower jaw (mandible) and the bone is moved forwards, backwards, sideways or rotated upwards depending on the deformity and then fixed with titanium plates and screws in the new position.

3. Double jaw surgery – It is a combination of the above procedures which involves surgery on both maxilla and mandible.

4. Genioplasty – In this operation, cuts are made in the chin (central) region of the lower jaw and the bone moved forwards, backwards, upwards or downwards depending upon the deformity.

Any tests or preparation required?

You might have to undergo an OPG, which is a specialized X-ray of the jaw, a Lateral Cephalogram and a CT scan to plan the surgery. Planning of the surgery is done through specialized softwares using Virtual Surgical planning, which helps in preparation of models and splints required for the surgery. Dr Vybhav Deraje collaborates with Osteo3D, which is a Bangalore based startup, for virtual surgical planning and preparation of models and splints. Removal of all wisdom teeth might also be required in most cases. Surgery will be usually preceded by 6 months to 1 year of Orthodontics and followed by 3months to 6 months of Orthodontics. Surgery first approach, where the surgery is done first and then followed by Orthodontics is also available and you can discuss that option with your surgeon during your consultation.

What does the surgery involve?

You will be seen by our anaesthetist before surgery and then admitted on the day of surgery. The surgery is conducted under general anaesthesia and takes about 2-3 hours. The cuts are made on the upper and lower jaw depending upon the surgery and the bones are brought to the correct position and fixed with Titanium plates and screws. Dr Vybhav Deraje uses Matrix plating system and Synthes plating system for all his cases. All the incisions are inside the mouth. So there are no external scars. All the wounds are closed with absorbable stitches. In some cases, a blood transfusion might be required.

What happens after surgery?

You will be usually kept in ICU for one day and shifted to the ward the next day. You might notice some mild oozing of blood from the mouth which is normal. There will be considerable swelling which slowly recedes over the week. You will be on soft diet for 3 weeks. You will be advised to give regular mouthwashes to keep the mouth clean. You will be discharged after 3-4 days. There will be some rubber bands applied to the braces on discharge. There will be a follow up appointment after 1 week to check the wound. All the stitches are absorbable and you might notice them either getting absorbed or falling off in the subsequent weeks. Once the wound is healed, you will continue with some more orthodontics for 3 months to 6 months.

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What complications can I expect?

Infection, bleeding, swelling and pain are common complications that can happen. The pain will be managed by our acute pain service team while in the hospital. You might notice some numbness in the upper lip and lower lip due to stretching of the nerves. It is temporary, but in some patients can take 6 months to 1 year to recover completely. Loss of teeth also is a rare complication of this surgery and some patients might need some dental rehabilitation if that happens.

I have a few more questions. What do I do?

Kindly write to us at connect@drvybhavderaje.com. We will be happy to reply to any of your questions and concerns.

Disclaimer: This information sheet is for you to get a general idea about the condition and surgery. This is in no way a substitute for a formal consultation with your doctor.

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